NHSGGC PUBLIC PROTECTION SERVICE

‘Pathway for Neonatal Services when there are concerns regarding risk of abuse or neglect’

Lead Manager	Melanie Hutton, General Manager
Responsible Director	Jamie Redfern, Director of Royal Hospital for Children
Date approved	September 2025
Date for Review	September 2027
Author	Dr Alex Christmas, Doctor in training Paediatrics Dr Owen Forbes, Consultant in Child Protection
Number	1
Replaces previous version (if applicable)	

Pathway of care for infants under the care of Neonatology when concerns regarding risk of physical abuse and / or neglect are identified

Infant admitted to NNU or postnatal ward and a concern about potential for abuse/neglect is identified

↓

Consider whether there is a medical cause which requires investigation and treatment

↓

DOCUMENT concern in version of notes being used currently (most likely physical depending on unit/local practice)

EXPLAIN concern To Parents / Carers and action being taken

Make a telephone referral to **SOCIAL WORK** (see below)

Complete and send a **NOTIFICATION OF CONCERN*** and email to child's local area SW

**A copy is sent to CP admin at RHC but this does NOT initiate a referral to the Child protection team*

↓

Life Threatening Injuries

Medical management of the child takes PRIORITY.

When there is a significant concern of physical abuse, contact:

- **POLICE** Scotland directly via #101 or #999
- the Child Protection Consultant on call

↓

Referral to Child Protection Service

Mon – Fri – 09:00 -17:00 – Contact CPS advice line (86605)

OOH - Contact the Child Protection Consultant on call via Switchboard (you will need to state to switchboard that you wish to speak to consultant) – ***Routine referrals should be made in daytime hours***

Guidance Notes for Child Protection Pathways

The aim of this guidance is to provide a pathway of care for all staff looking after newborn infants in postnatal wards or neonatal units across Greater Glasgow and Clyde in which an injury has been identified and/or there is a concern about physical abuse or neglect. The purpose is to provide staff with an easy to access guideline to inform how to manage these cases.

It is the responsibility of clinical staff within NNU to recognise and act on child protection concerns identified at or during the child's admission to NNU. In some cases, concerns may already have been raised antenatally and so plans may already be in place for a post birth case conference or alternative discharge destination for the baby (kinship care etc). The first priority will always be the medical safety and stability of the child. It is important the family are kept informed of any ongoing child protection process and it is the responsibility of the NNU team to ensure that parents are updated of social work and Child Protection referrals and made aware of what these entail.

Where a baby is admitted to a neonatal unit out of hours, concerns regarding child protection can usually be actioned the following day, unless there are life threatening injuries and/or concerns regarding the safety of any other children in the family. If there are concerns about other children in the household, this would involve discussion with social work, as it is their responsibility to ensure other children are in a place of safety.

Discussions with parents

If you feel you need further advice and support about a case it is appropriate to speak to the CP team. Provided there is no NOC completed, you do not have to tell the family you have had this discussion. But there should be clear documentation in medical and / or nursing notes of any discussion that you have had with CP team.

If a NOC has been submitted, parents should be informed and next steps explained. There may be rare exceptions to this, for instance if it was felt that the risk of household violence or harm would be exacerbated if a family member was aware of the referral. It would be worth discussing next steps with SW +/- CP team if you felt this was the case

Child Protection Medical Assessments

In cases where a NNU clinician has identified a concern of abuse or neglect in a patient, there is not an expectation on this clinician to complete forensic examination of the child. It is the expectation that in cases where it is not clear whether a child protection concern has been identified that the NNU clinician complete a general history and examination to establish whether they are or are not concerned.

In cases of suspected physical abuse or neglect where the child or young person has been admitted to neonatal intensive care, the Child Protection Consultant will attend to complete the child protection medical proforma and perform a forensic examination. This will require further consideration if the child is not at RHC as depending on the case/circumstances, transfer of the infant may be needed – this should be discussed on a case-by-case basis. This will usually be in daytime hours, unless there is imminent concern that the child will not survive. Prior to the CP Consultant attending the ward, it is the expectation that the hospital team will have shared relevant information with social work and police if necessary. It is not the role of the Child

Protection Service to make initial referrals to social work. Following a child protection medical assessment, the Child Protection medical team will take primary responsibility for sharing of information and input into multi agency process. Responsibility for updating police and / or social work on the medical condition of the child will remain with the hospital clinicians who are managing the presentation.

The Child Protection consultant will be responsible for completing a medical report which will be stored in the patient's medical notes and shared with other agencies as part of the Child Protection Process. It is not the role of NNU clinicians to give opinions in relation to likelihood of inflicted injury. Questions in relation to intent or possible inflicted injury from statutory agencies should be redirected to the Child Protection team.

Considering a Medical Cause

It is important that consideration is given to a medical cause for any presentation and that medical investigations are instigated at presentation if required, and that relevant specialties review the child's injuries where appropriate. Child maltreatment should be considered within differential diagnosis.

Newborn falls/drops

There is a BAPM guideline that can be found below regarding the acute assessment and management when an infant has been dropped/has fallen on the postnatal ward. It should be noted that this guideline focuses on the acute medical management.

The Prevention, Assessment and Management of in-Hospital Newborn Falls and Drops | British Association of Perinatal Medicine

With regards to the need for discussion with the child protection team, these infants should be considered on a case-by-case basis.

In cases of unwitnessed injury, risk factors that may influence decision-making regarding escalation to the child protection team could include:

- The response by parent/caregiver to the incident e.g. was disclosure delayed?
- Prior or ongoing social work involvement e.g. has there been a pre-birth case conference, or is a post birth case conference planned?
- Any concerns raised from interactions on the postnatal ward
- Background of poor parental mental health or addiction
- Background of domestic/gender based violence
- Pattern of injury not felt to be consistent with reported mechanism

Life Threatening Injuries

If a child is admitted to NNU with life threatening injuries and there is a concern raised that the child has sustained these injuries through physical abuse, the Child Protection consultant should be contacted. They will attend NNU (if the child is medically stable enough) to complete a child protection medical assessment including documentation of injuries, but the timing of this may vary if contacted out-of-hours. If there is an

immediate concern the infant may pass away, the consultant will attend out-of-hours, but otherwise this would take place the following day.

If a child has sustained a life threatening accidental injury and concerns have been raised around vulnerability or neglect, the CP service should be contacted. The CP consultant may or may not attend to examine the child; this will depend on circumstances around admission and will be managed on an individual case basis.

Other concerns/presentations

It may be identified that a family has significant vulnerability from financial/social perspectives (for example asylum seekers). In these instances, it may be appropriate to consider discussing the family with the health visitor, with social work, and considering completing a NOC in a supportive manner to assist with housing, finances etc. It is still important in these cases to let the family know you have done this.

As infants get closer to discharge, there may be circumstances where parents are not adhering to medical advice, in a manner felt likely to be deleterious to the child on discharge. It is important to recognise that failure to meet the basic healthcare needs of a child is a form of neglect, but also that parental opinions about what is best for their child can differ from those of healthcare professionals for a variety of reasons. In these cases it may again be appropriate to seek advice and consider a NOC.

Social Work Referrals

It is the responsibility of the clinician who first assesses the infant and has noted a concern of abuse or neglect to raise a formal notification of concern with social work. When there is a high suspicion of physical or sexual abuse social work should be contacted by telephone at point of admission. Within office hours if the child is not known to an area team, a referral should be made to RHC social work (Extension 86394). If the child is known to an area team (has an allocated social worker), then this team should be contacted directly. Out of hours, referrals should be made to Health and Social Care Connect (01412870555 in hours, 03003431505 OOH). Children who live outside of Glasgow City will require contact with local social work teams.

It would be the expectation that the base hospital would have completed this process for children who are being transferred from other health boards. However if this has not been clearly documented on transfer letter or verbally communicated to transfer team, it will be necessary to ensure that this process has occurred during admission to NNU.

Please refer to the NHS GG&C NOC guideline for further information on how to complete a notification of concern and for social work contact details. (CLICK HERE FOR NOC GUIDANCE). Social work contact details have also been included in an appendix at the end of this pathway.

Multi Agency Working

It is the expectation that updates to statutory agencies (police and social work) in relation to on-going medical condition of the child or young person will be provided by attending inpatient clinicians. The Child Protection Service will lead on attending multi agency meetings, convening IRDs and provide opinion in the form of a CP report if requested to do so by these agencies.

Appendix 1 - Social Work Area Teams Numbers

Typically SW Offices are open

- Monday to Thursday inclusive: 8.45 am - 4.45 pm
- Friday: 8.45 am - 3.55pm

Out with these times staff should contact Glasgow and Partners Emergency Social Work Service (Standby) on 0300-343-1505

Glasgow City	
Glasgow Social Care Direct	0141 287 0555
Renfrewshire	
Paisley	0300 300 1199
Johnstone	
Renfrew	
East Renfrewshire	
Clarkston	0141 577 8300
Barrhead	
East Dunbartonshire	
Kirkintilloch	0141 777 3000
West Dunbartonshire	
Clydebank	0141 562 8800
Dumbarton/Alexandria	01398 608080
Inverclyde	
Greenock	01475 715365
Port Glasgow	01475 715270

Appendix 2 - Secure email addresses for Children and Families teams to be used for email of child protection notification of concern forms

Glasgow City	scdchildrenandfamilies@glasgow.gov.uk
Vale of Leven/Dumbarton	dumbarton.valeduty@west-dunbarton.gov.uk
Clydebank	clydebank.duty@west-dunbarton.gov.uk
Renfrewshire	childrenandfamilies@renfrewshire.gov.uk
East Dunbartonshire	childcarestandby.referrals@eastdunbarton.gov.uk
East Renfrewshire – Barrhead area	barrheadadmin@eastrenfrewshire.gov.uk
East Renfrewshire – Clarkston area	Clarkston.admin@eastrenfrewshire.gov.uk
Inverclyde	Childcare.Operations@inverclyde.gov.uk
Hospital Based Staff – QEUH/RHC and PRM	RHCHospital@glasgow.gov.uk